

My personal action plan (conventional-non-MART)

Name:	Given Name Surname	Height:	Single Code Entry: Standing height	Weight:	Single Code Entry: Body weight
Date of birth:	Date of Birth	Best Peak Flow:		Predicted Peak Flow:	
NHS number:	NHS Number	Allergies:		Triggers:	
Hospital number:	Hospital Number				
GP Name	Usual GP Full Name		Tel:	Organisation Telephone Number	
Practice Nurse name:			Tel:		
Hospital Practitioner			Tel:		

My asthma treatment

Medication / Inhaler Colour	Strength	Dose and Frequency	Device / Type of Medication
Issued by:		Date:	

The Four Stages of Asthma Control

Peak flow recorded from 5 years and above.

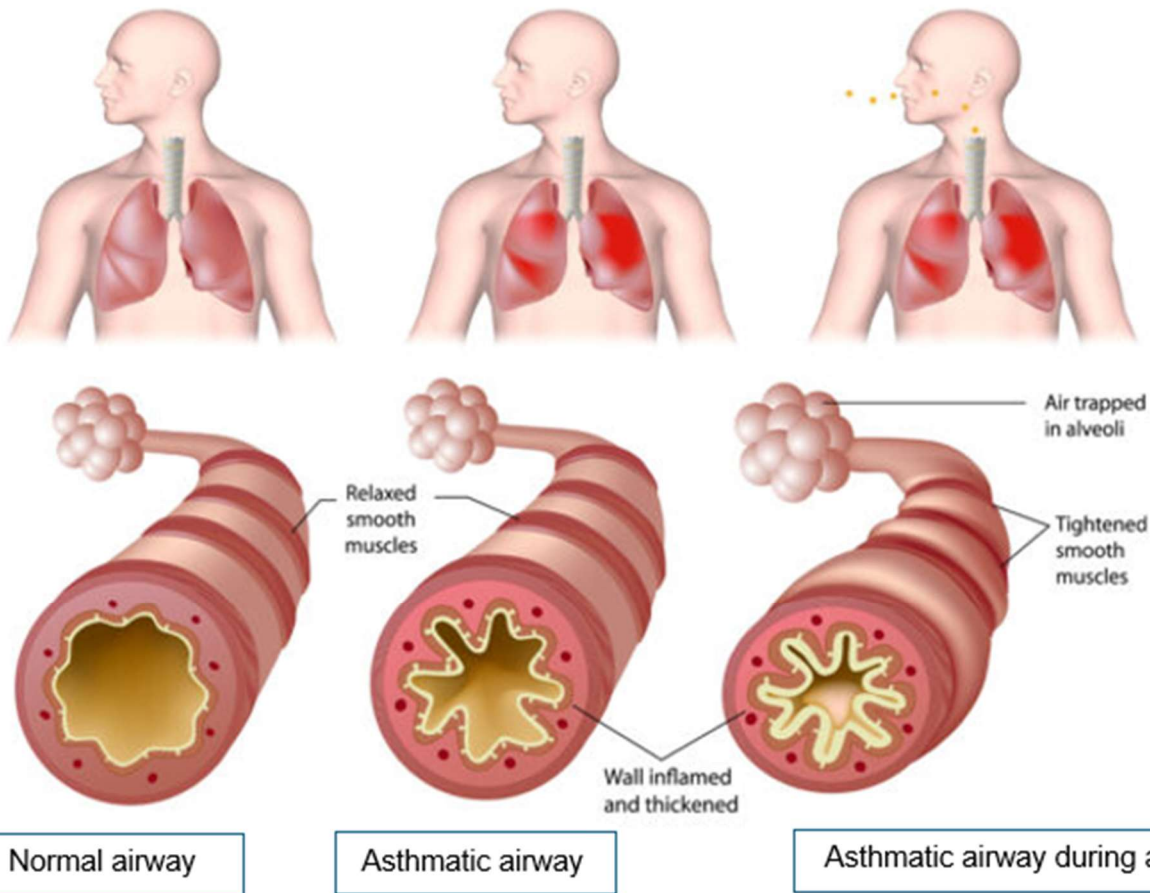
Stage 1	Key messages	Recommended Actions
Your asthma is under good control when: - There is no cough, wheeze, chest tightness or shortness of breath. - You are able to do usual activities. - Your peak flow is 75% -100% of best (5 years and above). 100% 75%	Remember: Continue to take preventer medication(s).	Take your preventer treatment as prescribed to keep you well. Preventer medication (s): Attend a planned asthma review once or twice per year.

Stage 2	Key messages	Recommended Actions
Your asthma control is not good when you have any of the following: - Cough, wheeze, chest tightness or shortness of breath. - Waking at night more than once per week due to asthma symptoms. - You are able to do some, but not all usual activities. - Your peak flow is 60%-75% of best (5 years and above). 75% 60%	Important: Seek a medical review (GP/111) if reliever (blue) inhaler is: - Needed three times a week or more. - Needed regularly for more than 24 hours	For reliever (blue) inhaler Take 2 puffs via MDI and spacer device (leaving 30-60 seconds between each puff). If no relief after 5-10 minutes take another 2 puffs (one puff every 30-60 seconds) you have now had 4 puffs. This should make you feel better and your symptoms should have improved. If your symptoms are not improving move to stage 3.

Stage 3	Key messages	Recommended Actions
You are having an asthma attack when: - Symptoms are more frequent. - Reliever is less effective and is needed more often than usual. - Reliever is not lasting 4 hours. - You are breathless on normal activity. - Your peak flow is 60% or less (5 years and above). 60%	Important: Seek a medical review (GP/111) within the next 24 hours if: - You have needed to use 6 puffs of your reliever (blue) medication on a single occasion even if you feel better. - Reliever (blue) inhaler is not lasting at least 4 hours. - Reliever (blue) inhaler medication is needed regularly for more than 24 hours.	For reliever (blue) inhaler Take another 2 puffs via MDI and spacer device (leaving 30-60 seconds between each puff) you have now had 6 puffs. If your symptoms have now improved still arrange a medical review. You may need to start a course of prednisolone If your symptoms are not improving move to stage 4.

Stage 4	Key messages	Recommended Actions
You are having a severe asthma attack when: - Symptoms continue to get worse. - Symptoms are not responding to reliever inhaler - You are too breathless or exhausted to speak. - Peak flow is 40% or less (5 years and above). 40%	Important: Arrange an immediate medical review. Call 999 immediately if there is no or poor response to your reliever blue inhaler.	Try to keep calm: Take 4 more puffs of your reliever (blue) inhaler via MDI and spacer device (one puff every 30-60 seconds). You have now had 10 puffs. Wait 5-10 minutes: if symptoms have improved still arrange an urgent medical review. If symptoms are not improving immediately call 999. If the ambulance has not arrived after 10 minutes and your symptoms are not improving call 999 again and repeat taking up to 10 puffs of your reliever (blue) inhaler (one puff every 30-60 seconds).

Asthma and Your Airways



Handy Tips

Do not run out of your inhalers!

- Each inhaler comes with a set amount of doses. Keep a check on the expiry date and dose counter or count out the doses if no dose counter to avoid running out of treatment. Once empty it will only deliver the propellant.
- Always carry your dry powder device/pMDI and spacer with you including at school and work.

Inhaler maintenance

- Rinse your mouth and gargle after using steroid Inhalers.
- Spacers need to be washed once weekly in warm soapy water and allow to air dry.
- If you use a spacer, replace this every 6 to 12 months.
- Access www.asthmaandlung.org.uk to check your inhaler technique
- If your reliever medication is needed regularly three times a week or more, arrange a medical review.

Asthma Review

- Please bring all of your medications and personal asthma action plan with you if you are admitted to hospital or if you attend any outpatient appointments.
- Please arrange a review with your GP, or asthma nurse within two working days after an admission to hospital or after receiving emergency care.
- If you have commenced a rescue course of prednisolone, you should contact your GP for a review.
- You will need to attend your asthma reviews and take your medication as directed.